

디자인 사양 (시방서)		대원제약
작성자 / 신규 작성일	이규왕	2018.11.12
제품명	메게스아이 설명서-말레이시아	
규격 (장폭고)(mm)	100 x 285 (mm)	
인쇄도수	양면 1도	
용지 (g)	모조 70g	
코팅 / 후가공	-	
개정내용	신규	
개정번호 / 개정일자	V.0000	2018.11.12

MEGEX-I Suspension 800mg/20ml

DESCRIPTION

White to cream coloured suspension with orange fragrant and taste.

CONTENTS

Each 20ml of suspension contains megestrol acetate 800mg.

Preservative: Sodium benzoate 0.1% w/v

PHARMACODYNAMICS

Megestrol acetate exhibits classical progestational activity. It possesses little or no estrogenic, androgenic or adrenocorticoid action. The anti-tumour action of megestrol acetate on carcinoma of the breast is unclear. However, it is known to compete for progesterone, androgen and glucocorticoid receptors, and affect pituitary functions.

Megestrol acetate is even used to enhance appetite in patients with cachexia and anorexia. The precise mechanism of this is unknown but the efficacy of megestrol acetate in cachexia and anorexia has been established with statistically significant improvement in appetite. The gain in weight while taking megestrol acetate, particularly at high doses, is usually not associated with water retention, but is associated with an increased appetite/food intake and an increase in fat and body cell mass.

PHARMACOKINETICS

Mechanism of Action

The appetite enhancing property of megestrol acetate and its possible use in cachexia have been recorded. However, the exact mechanism on how megestrol acetate produces effects in anorexia and cachexia remain unknown.

Absorption and Distribution

The oral absorption of megestrol acetate appeared to be variable. Estimates of the plasma concentrations of megestrol acetate are dependent, not only on the measurement method used, but also on intestinal and hepatic inactivation of the drug, which may be affected by intestinal tract motility, intestinal bacteria, antibiotics administered, body weight, diet, and hepatic function. There is no difference in safety following administration of megestrol acetate under fasting and fed conditions, therefore megestrol acetate could be taken without regard to meals.

Metabolism and Excretion

Metabolites have accounted for only 5% to 8% of an administered dose of megestrol acetate. The administered dose is usually excreted in the urine (approx. 66%) and faeces (approx. 20%). Respiratory excretion and fat storage may account for the fraction of an administered dose not found in urine or faeces. The half-life elimination of megestrol acetate ranged from 20 to 50 hours.

Specific Populations

The pharmacokinetics of megestrol acetate has not been studied in paediatric, renal impairment, and hepatic impairment.

The effect of food on the bioavailability of megestrol acetate has not been evaluated.

INDICATION

Megex-I Suspension 800mg/20ml is indicated in male and female patients

for the treatment of anorexia or weight loss secondary to cancer or acquired immune deficiency syndrome.

DOSAGE & ADMINISTRATION

Anorexia or weight loss: 400 - 800 mg given as a single daily dose. Shake the suspension container well before using. Can be taken directly from the pouch. A plastic dosage cup with 10 ml markings is provided for convenience.

At least two months of continuous treatment is considered an adequate period for determining the efficacy of megestrol acetate.

MODE OF ADMINISTRATION

Oral administration

CONTRAINDICATIONS

- Patients who are hypersensitive to megestrol acetate and/or the excipients of this product.
- Women who are pregnant or have suspected pregnancy.
- Lactating women.

WARNINGS & PRECAUTIONS

Megestrol acetate should be used with caution in patients with history of thrombophlebitis and in patients with severe impaired liver function.

Clinical studies of megestrol acetate did not include sufficient numbers of patients aged 65 years of age or older to determine if they respond differently than younger patients. Other reported clinical experience has not identified differences in responses between elderly and younger patients. Dose selection for an elderly patient should be cautious, usually starting at the low end of the dosing range, reflecting the greater frequency of decreased hepatic, renal or cardiac function, and of concomitant disease or other drug therapy.

Megestrol acetate is known to be substantially excreted by the kidney, and the risk of toxic reactions to this drug may be greater in patients with impaired renal function. Because elderly patients are more likely to have decreased renal function, care should be taken during treatment with megestrol acetate, and it may be useful to monitor renal function.

WARNINGS: The use of progestational agents during the first four months of pregnancy is not recommended.

Progestational agents have been used, beginning in the first trimester of pregnancy, in an attempt to prevent habitual abortion or treat threatened abortion. There is no adequate evidence that such use is effective and there is evidence of potential harm to the foetus when such drugs are given during the first four months of pregnancy. Furthermore, in the vast majority of women, the cause of abortion is a defective ovum, which progestational agents could not be expected to influence. In addition, the use of progestational agents, with their uterine-relaxant properties, in patients with fertilized defective ova may cause a delay in spontaneous abortion. Therefore, the use of such drugs during the first four months of pregnancy is not

recommended. Women of childbearing potential should be advised to avoid becoming pregnant. Megestrol acetate is a progesterone derivative, vaginal bleeding may occur in women who take this drug.

The glucocorticoid activity of megestrol acetate has not been fully evaluated. High blood sugar (hyperglycaemia) and worsening of diabetes and Cushing's syndrome may occur. This drug may decrease adrenal gland activity; adrenocorticotropin (ACTH) stimulation testing has revealed the frequent occurrence of asymptomatic pituitary-adrenal suppression. The possibility of adrenal insufficiency should be considered in any patient who presents with symptoms and signs suggestive of hypoadrenalism (e.g. hypotonia, nausea, vomiting, dizziness or weakness). Failure to recognize inhibition of the hypothalamic-pituitary-adrenal axis may result in death, so consideration should be given to the use of empiric therapy with stress doses of a rapidly acting glucocorticoid.

Effects on ability to drive and use machines

There are no known effects of megestrol acetate on the ability to drive or operate machinery.

INTERACTION WITH OTHER MEDICAMENTS

The possible interactions of megestrol acetate with concomitant medications have not been investigated. However, the decreased efficacy of some progestins has been suggested to be caused by enhanced metabolism of progestins by hepatic enzyme inducing medications. These include carbamazepine, phenobarbital, phenytoin, rifabutin or rifampin. Phenytoin and rifampin increase the serum concentrations of sex-hormone binding globulin (SHBG), significantly decreasing the serum concentration of free drug for some progestins.

Pharmacokinetic studies show that there are no significant alterations in pharmacokinetic parameters of zidovudine to warrant dosage adjustment when megestrol acetate is administered with these drugs. The effects of zidovudine on the pharmacokinetics of megestrol acetate were not studied.

Due to the significant decrease in the exposure of indinavir by megestrol acetate, administration of a higher dose of indinavir should be considered when co-administering with megestrol acetate.

PREGNANCY & LACTATION

Use in pregnancy

- This medication must not be used during pregnancy. Megestrol may cause harm to your unborn baby.
- Women of child-bearing age should use an effective form of birth-control while using this medication.

Use in lactation

Megestrol may pass into breast milk and could have undesirable effects on a nursing infant. Therefore, breast-feeding is not recommended while using this drug.

ADVERSE REACTIONS

- Cardiovascular System: Hypertension, cardiomyopathy, chest-pain,

palpitation, oedema and peripheral oedema, thrombophlebitis, pulmonary embolism (in some cases fatal), cardiac failure

- Digestive System: Diarrhoea, flatulence, vomiting, nausea, dyspepsia, abdominal pain, constipation, oral moniliasis, dry mouth, increased salivation
- Endocrine & Metabolic: Hyperglycaemia, decreased libido, increased lactate dehydrogenase, adrenocortical insufficiency, amenorrhoea, Cushing's syndrome, diabetes mellitus, hot flash, HPA-axis suppression, hypercalcaemia, weight gain (not attributed to oedema or fluid retention)
- Haematologic & Oncologic: Leukopenia, sarcoma, tumour flare (with or without hypercalcaemia)
- Hepatic: Hepatomegaly
- Infection: Candidiasis, herpes virus infection, infection
- Nervous System: Headache, pain, insomnia, paraesthesia, confusion, convulsion, depression, neuropathy, hypaesthesia, abnormal thinking, carpal tunnel syndrome, lethargy, malaise, mood changes
- Ophthalmic: Amblyopia
- Respiratory System: Dyspnoea, cough, pharyngitis, pneumonia, hyperventilation and lung disorder
- Skin and Appendages: Skin rash, alopecia, herpes, pruritus, vesiculobullous rash, sweating and dermatological disorder
- Urogenital System: Impotence, albuminuria, urinary incontinence, urinary tract infection, gynaecomastia, urinary frequency, breakthrough bleeding, menorrhagia, erectile dysfunction
- Neuromuscular & Skeletal: Weakness
- Miscellaneous: Fever

SYMPTOMS AND TREATMENT OF OVERDOSE

- No serious unexpected side effects have occurred from studies involving megestrol acetate administered in dosages as high as 1600 mg/day in a period of 6 months or more.
- Due to its low solubility it is postulated that dialysis would not be an effective means of treating overdose.

STORAGE CONDITIONS & PHARMACEUTICAL PRECAUTIONS

Store below 30°C in a tight container, protect from heat and light. Keep out of the reach of children. Jauhi daripada kanak-kanak.

SHELF-LIFE: Please refer to the outer box label

PACKING/PACK SIZES

Box of 20 sachets of 20ml

MANUFACTURED BY

DAEWON PHARM. CO. LTD.
24, Jeyakongdan 1-gil,
Hyangnam-eup, Hwaseong-si,
Gyeonggi-do, Republic of Korea.

Revision date: Oct 2018